

Transforming Certification for Better Care

ABMS CONFERENCE 2026

ABPN Article-Based Continuing Certification (ABCC)

Program Design, Current Participation, Performance, and User Experience

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Disclosure

Employment: American Board of Psychiatry and Neurology

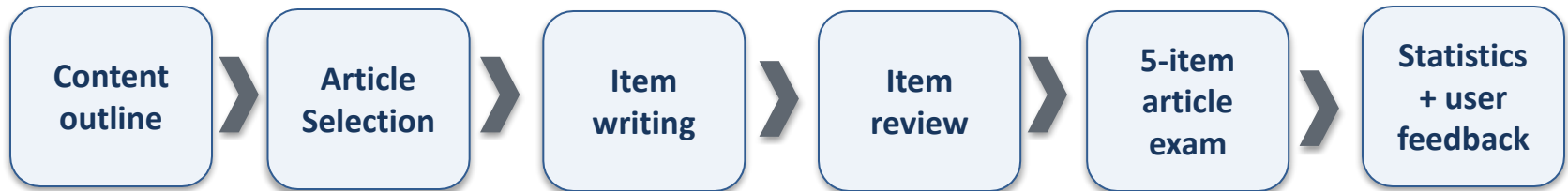
Conflicts of interest: None to disclose

ABPN Article-Based Continuing Certification Pathway

- Online alternative to the traditional 10-year Continuing Certification examination
- Integrates assessment with lifelong learning, clinical relevance, flexibility, and feedback
- 60 articles per specialty/subspecialty across 10 categories (6 per category)
- Diplomates select articles, with a maximum of 4 per category
- Complete 20 article exams per 3-year assessment block; up to 27 attempts allowed
- Each exam has 5 questions; 4 of 5 correct required to pass
- Operates alongside other Continuing Certification requirements



ABCC Program Design and Operational Framework



Dedicated physician committees develop content outline using ABPN-defined specifications.

Committee select the articles that will become the focus of item writing

Committee members write and review ABCC items without answer keys, using the source article to simulate the diplomate experience. Questions may directly assess the article, such as comprehension and analysis of findings, and critique of study limitations, or assess application of the article's learning to clinical practice.

Operational review continues after launch: item statistics and diplomate feedback are reviewed together.

Participation: ABCC Is the Predominant Assessment Pathway

55,293

Diplomates in Article Pathway
92.3% of 59,885
diplomates

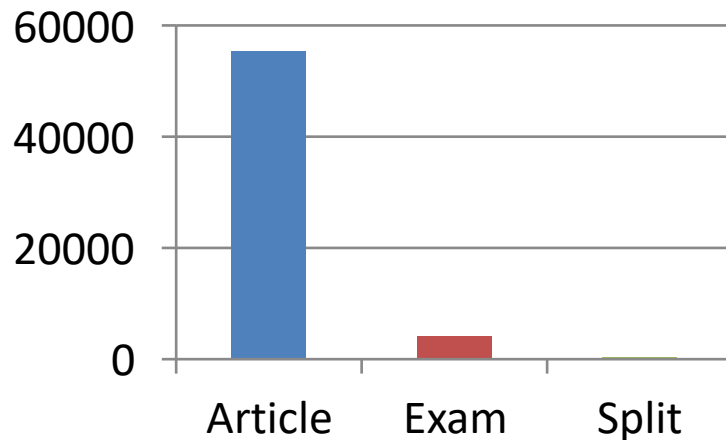
77,287

Certifications in Article Pathway
92.9% of 83,186
certifications

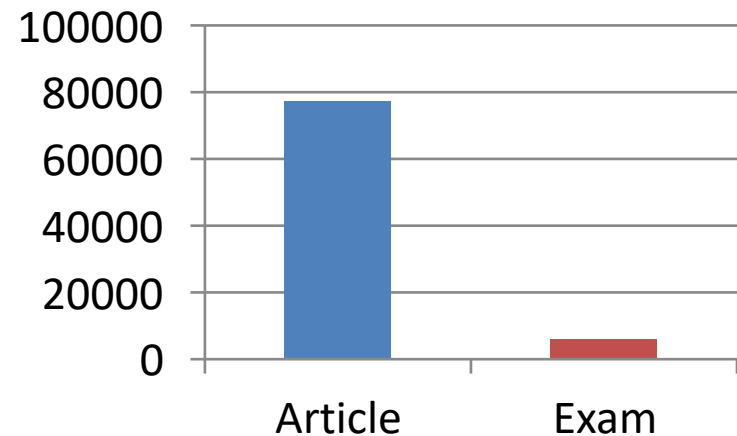
5,899

Certifications in Exam Pathway
7.1% of certifications

Diplomates by Pathway

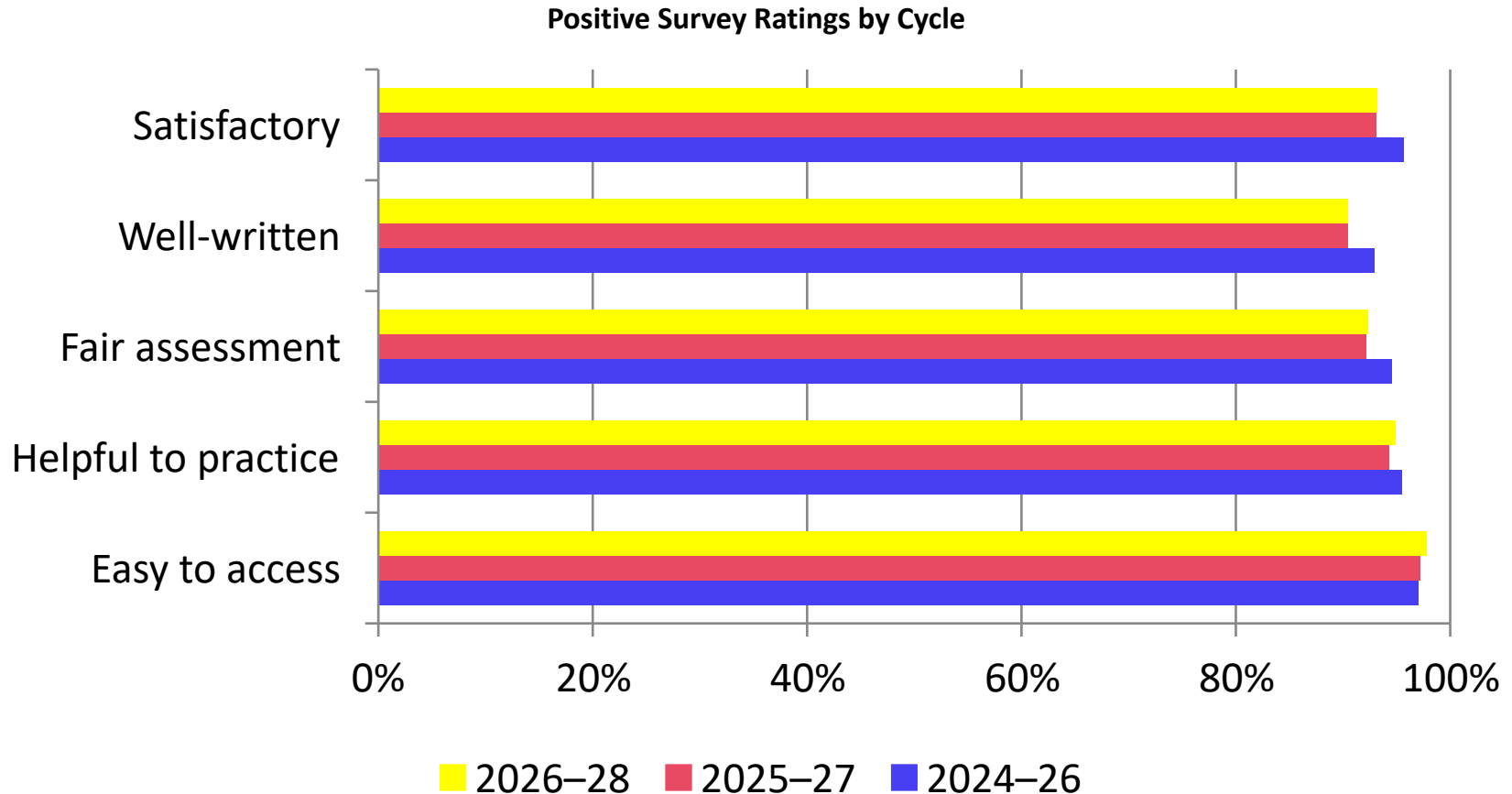


Certifications by Pathway



User Experience: Satisfaction Remains High

Percent Strongly Agree + Agree



Across all three cycles, approximately 90% or more responded positively to every satisfaction item.

Article access receives the strongest ratings (97.0%–97.7% positive).

What Users Tell Us: Relevance, Fairness, and Question Quality

94.3–95.4%

Article helpful to practice
positive across 3 cycles

92.2–94.6%

Fair assessment
positive across 3 cycles

90.5–92.9%

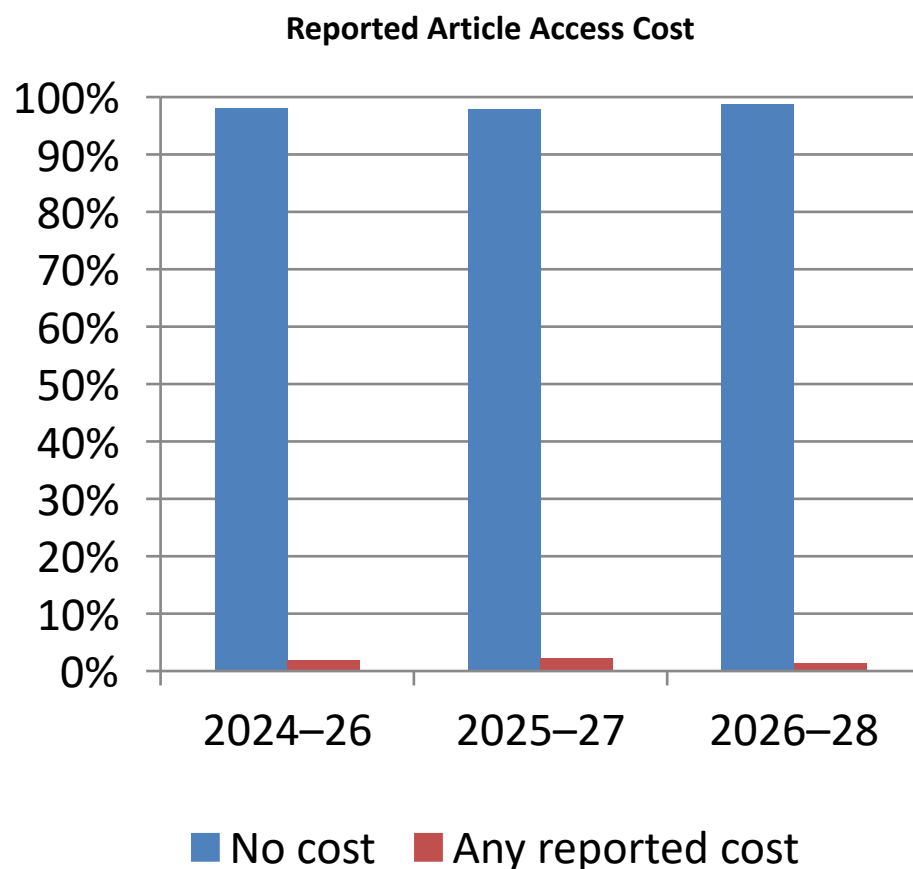
Questions well-written
positive across 3 cycles

The dashboard supports a consistently favorable user experience at very large response volumes (18,531 to 175,068 responses per cycle/question).

Question-writing is the lowest-rated of the five domains, although still above 90% positive in each cycle—an important target for continued quality improvement.

This reinforces the operational approach of combining psychometric review with direct diplomate feedback.

Article Access and Cost: Minimal Direct Cost for Most Users



97.8–98.6%

reported no cost
across current cycles

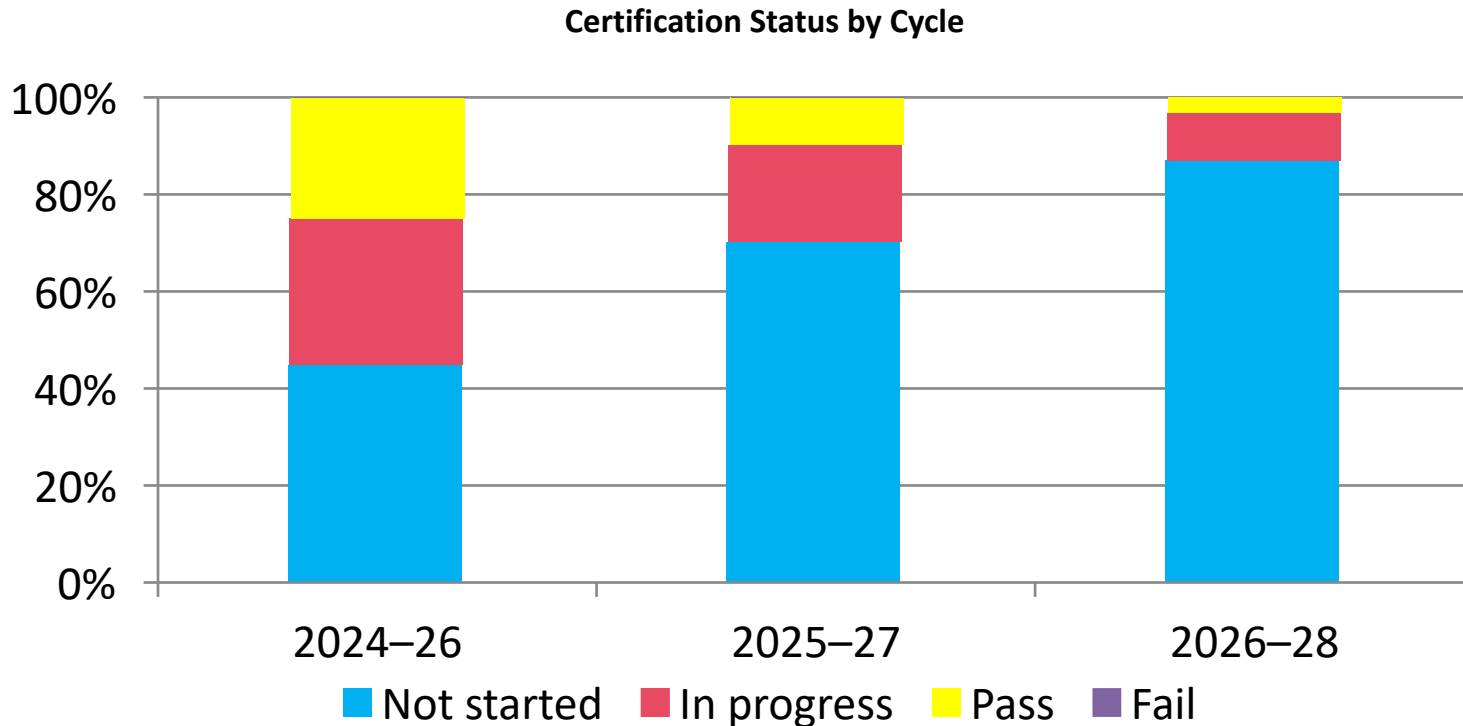
≤0.5%

in most paid bands
per cycle

The current dashboard suggests that direct article cost is uncommon for participants.

Access can still depend on journal subscriptions, professional society membership, institutional libraries, or changing publisher access policies.

Performance / Status Across Current ABCC Cycles



Cycle position matters: newer cycles naturally contain a larger proportion that have not yet started.

Across all active cycle records: 71,739 certifications; 6,465 have already passed and only 8 are currently recorded as failed.

Key Takeaways

ABCC combines a standardized assessment framework with meaningful physician choice in article selection and timing.

Current adoption is high: 92.9% of certifications and 92.3% of diplomates in the dashboard are in the Article Pathway.

User experience is strongly favorable: roughly 90%–98% positive across access, relevance, fairness, question quality, and satisfaction measures.

Article access is generally low-cost: approximately 98% report no direct cost.

Once the 2024–2026 cycle is complete, the next analytic layer should connect item/article characteristics and physician subgroups to performance while continuing to use user feedback for quality improvement.